



Accounting for the ‘dark side’ of new organizational forms: The case of healthcare professionals

Graeme Currie, Rachael Finn and Graham Martin

ABSTRACT

Our study responds to a call for research to integrate institutional, organizational and individual levels of analysis in examining the implications for work and employment relations of new organizational forms. We empirically examine the implementation of network forms of genetics healthcare delivery that cross organizational and professional boundaries. We highlight that less powerful professional groups may find difficulty in enacting boundary-spanning roles associated with new organizational forms. This is due, first, to inconsistency of government policy, which fragments organizations. Second, professional institutions sustain professional hierarchy and power differentials.

KEYWORDS

employment ■ healthcare organizations ■ human resources and industrial relations ■ institutional theory ■ public management

Introduction

Our study responds to a call for research to integrate institutional, organizational and individual levels of analysis in examining the implications for work and employment relations of ‘blurring organizational boundaries and disordering hierarchies’ (Marchington et al., 2005). To illustrate implications for work and employment relations of changing organizational forms, we

empirically examine the implementation of network¹ forms of healthcare delivery that cross organizational and professional boundaries.

Our study addresses the question: what work and employment relations challenges arise from the response of professionals expected to deliver services through networks?

Our article is structured as follows. First, we review literature that examines management of change associated with new organizational forms, focusing upon work and employment relations challenges. Linked to this, we outline a more specific literature that focuses on work and employment relations challenges associated with network forms of delivering public services. Having set out our research design and context, we present and discuss our data regarding challenges in enacting boundary-crossing roles for nurses associated with network forms of delivery from the perspective of professionals involved in the networks. The data are themed as follows: i) inter-organizational dimensions of the network challenge; ii) inter-professional dimensions of the network challenge. In conclusion, we highlight practical implications, our theoretical contribution and future research needs.

New organizational forms: Implications for work and employment relations

Focusing upon inter-organizational relationships, there are a number of key contributions in the organization studies and human resource management (HRM) literature which investigate a range of network forms and their implications for work and employment relationships (Kinnie et al., 2005; Marchington & Vincent, 2004; Marchington et al., 2005; Rainnie, 1991, 1992; Scarbrough, 2000). These emphasize the 'dark side' of networks and suggest organizational relationships are characterized less by collaboration, and more by exploitation of the less powerful by more powerful actors.

For example, Bachmann (2001, 2003) emphasizes that the institutional environment is shaping the quality of inter-organizational relations, with the potential for more powerful organizations in networks to dominate the relationship. For Bachmann, inter-organizational relations are controlled by patterns of trust and power, which are characteristic of the institutions in which business activities are embedded. Buttressing this assertion, in his analysis of supply chain relationships, Rainnie (1991, 1992) highlights unequal power relationships between large buyers and small suppliers with associated intensification of work pressures. Kinnie et al. (2005), in examining knowledge sharing within networks, also suggest an exploitative

dimension to inter-organizational relations that we need to better understand through examining the experiences of less powerful organizations participating in networks. Meanwhile, Scarbrough (2000) also notes the political nature of supply chain relationships with changes in work practices unilaterally imposed by more powerful organizations upon others. Relatedly, he suggests HRM practices cannot be readily co-opted into supporting and socializing new business processes around network forms of organizing and highlights an uncomfortable co-existence within supply chains of divergent organizing principles – markets, hierarchies and networks. Finally, he concludes network forms of organization, specifically supply chains, may be difficult to institutionalize.

Within Marchington et al. (2005), a number of contributions succinctly capture the institutional essence of work and employment relations challenges generated by changing organizational forms. In the introduction to the book, as a springboard for discussing challenges, Grimshaw et al. (2005a) also highlight that network forms of organizing are diffused within established market and hierarchical structures. Rather than transformation of organizations into 'new forms' or 'networks', Grimshaw et al. present evidence to show that changes frequently involve hybrid elements within established organizational forms that are complex and contradictory. Taking account of this, they highlight the dualities of building hierarchies and networks – for example, seeking greater performance accountability upwards and horizontal integration sideways – have implications for work and employment relations. In particular, employees are charged with building networks, whilst simultaneously occupying the cusp between order and disorder. In a later chapter, Grimshaw et al. (2005b) emphasize the way that network forms provide the basis for redistribution or consolidation of power among organizations and the relative gain for more powerful members of the network, compared to the less powerful. Meanwhile, Rubery et al. (2005) reinforce Scarbrough's assertion regarding the difficulty of implementing a coherent and consistent set of HRM policies and practices across networks due to divergent organizational objectives (Scarbrough, 2000). Specifically, employees positioned in boundary-crossing roles, face uncertainty about where the locus of control and authority lies. Rubery et al. argue that new forms of organization, such as networks, where inter-organizational relations are emphasized, increase the need to consider the employment relationship beyond the specific workplace or enterprise. Finally, in the concluding chapter, Grimshaw et al. (2005c) suggest greater attention be paid to how organizations are embedded within wider institutional structures.

Utilizing arguments within Marchington et al. (2005), we move on to consider the impact of wider institutional structures associated with public

services, within which networks forms of organizing healthcare are embedded, upon work and employment relationships. Evident within a more sociologically orientated public administration literature is the significance of professional boundaries, as well as organizational boundaries.

Organizational and professional boundaries in public services networks

Building upon the New Right critique of bureaucracy, with its election in 1997, the Labour Government in the UK pursued a policy agenda concerned with modernizing public services. Central to this was the idea of pursuing a 'Third Way', between hierarchies on the one hand and markets on the other (Hudson, 2004). This drew upon the rise of the network form of organization within the private sector (Castells & Hall, 1994; Nohria & Eccles, 1992; Reed, 1992) and converged with an emphasis upon network forms of organizing public services delivery across Europe (Kickert et al., 1997; Klijn et al., 1995; Scharpf, 1993) and the United States (Hall & O'Toole, 2000, 2004).

However, policy aspirations for network forms of delivering public services may be stymied, firstly by organizational fragmentation, which is engendered by other aspects of policy, notably marketization and an associated strong performance regime. As earlier discussed, there exists a strong possibility that hybrid networks or contradictory organizational forms (networks, hierarchy and markets) emerge following policy initiatives (Grimshaw et al., 2005a). Driven by market-based reforms, there exists tension regarding the presence and form of the state and its intervention at a local level. A tight financial relationship has been created between central government and local public services organizations. Those charged with leading public service organizations are forced to meet short-term targets and make efficiency savings demanded by central government (Lucio et al., 1997). Consequently, the extent to which public services organizations can move beyond a narrow economic agenda, with its organizationally focused inspection and audit regimes, is limited (Ferlie et al., 2003; Hood et al., 2000; McNulty & Ferlie, 2002; Newman, 2001). This has implications for new forms of organizing public services with simultaneous centralization and decentralization. Tensions within 'neobureaucratic' modes of organizing may militate against strategic programmes for change in the public sector anticipated by policy-makers (Lucio et al., 1997), with vertical 'control and command' structures difficult to supplant with partnership, networking and lateral modes of organizing (Ferlie et al., 2003).

A host of recent studies confirm the importance of appropriate incentives and sanctions within public sector settings, including healthcare, to shape organizational and individual behaviours within networks (Ferlie & McGivern, 2003; Fitzgerald et al., 2006; Goodwin et al., 2004; Hudson, 2004; Jackson & Stainsby, 2000). These studies emphasize that 'government targets, audit and incentive arrangements need to be harmonized to promote and reward working in networks in order to avoid the potential of existing and differing targets within individual organizations preventing effective partnership working and networking' (Goodwin et al., 2004: 4). In sum, government policy that simultaneously promotes networks and marketization, embedded in which is a strong performance management emphasis, might therefore be characterized as inconsistent (Currie & Suhomlinova, 2006; Newman, 2001).

Second, policy aspirations for network forms of delivering public services may be stymied by the professional institution. Public services organizations, exemplified by the case of healthcare, are characterized by a proliferation of professional boundaries. Power is not derived solely from position or hierarchy, but from professional knowledge. The jurisdiction over this knowledge domain is guarded assiduously. Commonly one's ability to practise requires a qualification or credential controlled by the relevant profession. In short, there exists a system of professions that acts to retard the emergence of boundary-spanning roles enacted by subordinate professionals (Abbott, 1988).

Not only is knowledge segmented laterally by specialization, but a professional logic of hierarchy is dominant, which remains essentially paternalistic and authoritarian (Bate, 2000). Within the healthcare setting, clinicians are at the apex of the organizational pyramid and are able to resist managerial interventions (Ackroyd, 1996; Ferlie & Pettigrew, 1996; Ferlie et al., 1996; Harrison et al., 1992). Meanwhile, the role of nurses has remained one of 'handmaidens' to clinicians. Harrison et al. (1992: 18), for example, described nursing as a profession,

shaped and conditioned by doctors, and where appropriate deference was for long a quite explicit part of the process of nurse socialization . . . The chief surgeon, with his carnation buttonhole, striding through the ward on a 'grand round', pursued by a gaggle of juniors and nurses, is clearly a star.

Such 'professionalization' has particular implications in relation to the way that roles are carried out and change is managed when implementing new organizational forms to deliver healthcare; that is, strong intra-professional

networks within medicine may co-exist with weak inter-professional networks between nursing and medicine (Fitzgerald et al., 2006).

In sum, professional hierarchy and traditional power relationships, combined with marketization and a strong centralized performance regime focused upon the organizational level, adversely impact upon policy aspirations for transcendence of professional and organizational boundaries through implementing network forms of organizing. Associated with this are significant work and employment relations challenges, which have not always been thought through by policy-makers and those charged with implementing networks (Ferlie & McGivern, 2003; Ferlie & Pettigrew, 1996; Goodwin et al., 2004; Hyde et al., 2005, 2006a, 2006b).

To re-iterate our intention, we address a significant research gap in understanding the work and employment relations challenges arising from networked product and service delivery in professionalized settings, through in-depth empirical work situated in healthcare.

Research design

The case method

We took a case study approach designed to engender analysis of work and employment relations challenges associated with the implementation of new organizational forms. We focused upon a government policy initiative: 'mainstreaming genetics'. Through 'mainstreaming genetics' the national government body, Department of Health (DoH), awarded funding for pilot genetics projects on the basis they encourage the development of networks to supplant traditional professional hierarchy, cross organizational boundaries (for example, between regional genetics centres and acute hospitals), and encompass an associated development of new roles. These roles are designed to shift the balance of power from specialist genetics clinicians located in regional centres to others, such as mainstream clinicians and nurses in acute hospitals (Martin et al., 2007). In essence, mainstreaming genetics is a translational science initiative that transfers knowledge from its points of development in academic research centres into wider use in practice through the development of new, more integrated organizational forms.

The overall aim is two-fold: first, to make better use of limited resources and so make efficiency gains (e.g. regional specialists would only see complex cases with 'cheaper' nurses delivering routine services, such as risk assessment through family histories); second, to ensure advances in genetics knowledge are converted into health services delivered to the public.

Following exploratory interviews with clinicians, who led projects funded under the mainstreaming genetics programme (27 in total), we focused upon 11 in-depth cases as representative of the total 27 pilots funded by the DoH under the mainstreaming genetics programme: for example, a mix of projects driven through a regional genetics centre, acute hospital and primary care organization; a mix of disease areas into which genetics was mainstreamed; projects deemed in exploratory interviews to be successful and experiencing significant problems. Within this article, we present analysis of the four in-depth cases, where delivery of genetics services crossed the regional genetics centre and acute hospital boundary: 'Carcity'; 'Unicity'; 'Shipcity'; 'Bicity'. We focus upon cases where the regional centre–acute hospital boundary was crossed on the basis that one of the main empirical and theoretical concerns lies with the interaction of nurses and clinicians. The relationship between these two groups differs in primary care settings, where the nurse works in a relatively more autonomous fashion from GPs. In contrast, in a hospital setting, traditionally the nurse is a 'handmaiden' to the clinician (Harrison et al., 1992).

For all four cases, the bulk of funding (c. £250,000 over two to three years for each case) supports the recruitment of one or more genetics nurse, expected to work within the acute hospital and enact an autonomous (compared to traditional more subservient role to clinicians), boundary-crossing role, to deliver a networked genetics service. Our study focuses upon this nurse role, since difficulties associated with its enactment exemplify the work and employment relations challenges of implementing networks in healthcare.

Cases vary along two dimensions: a mix of disease areas into which genetics was mainstreamed² and projects deemed in exploratory interviews to be successful and experiencing significant problems (Unicity and Shipcity were judged successful and experienced fewer problems than Bicity and Carcity). We were concerned to understand work and employment relations challenges in each individual case (within-case analysis) and understand differences and similarities between cases (across-case analysis) (Eisenhardt, 1989, 1991; Yin, 1994).

Genetics nurses were meant to work in collaboration with mainstream clinicians at consultant level and other mainstream nurses in joint clinics, so that any intervention in the mainstream disease area was integrated with genetics interventions. Simultaneously, genetics nurses were meant to develop and implement care pathways for patients that required an integrated mainstream/genetics intervention. Other stakeholders, besides the clinicians and nurses working with the genetics nurse in the mainstream disease area included: genetics clinicians and a genetic counsellor (in Carcity and Bicity

this was an ex-nurse up-skilled through formal accredited genetic counselling training; in Shipcity, this was someone with a science background followed by an MSc in genetic counselling; Unicity failed to recruit a genetic counsellor) located in the regional centre responsible for supporting the genetics nurse roles. In short, first, cases encompass an inter-organizational component between a regional centre and acute hospital. Second, cases encompass an inter-professional (between clinicians and nurses) and intra-professional (between mainstream clinicians and genetics clinicians; between nurses and genetic counsellors) dimension.

Use of interviews

Following exploratory interviews with clinical leads across the four cases to scope the dimensions of each case and identify early challenges in the development and implementation of networks, interviews were carried out across cases with a wide range of stakeholders (identified by the clinical lead as significantly engaged in the network). In total, we carried out 40 interviews across four cases with: genetics nurses; genetics clinicians; mainstream clinicians; mainstream nurse managers; business managers; project managers; commissioners of service.³

In analysing interviews within the four cases, a number of iterations were undertaken by the authors in the development of key themes. The themes reflected healthcare professionals' perceptions of work and employment relations challenges within their network. Rather than coding the interviews through software, such as QSR's NVivo package, so that data remained contextualized we treated the interviews holistically as narratives. Interviewees were telling stories relating to their experiences, typically time sequenced, that chronicled the emergence of work and employment relations challenges over the course of changes to the delivery healthcare through the network (Reissman, 1993). In this vein, the three authors read interview transcripts and then created analytic notes alongside the interview transcript, which were then copied and placed within the accompanying interview text; that is, embedded analysis. Related parts of the embedded analysis in each interview were placed together. Each of the three authors had engaged in fieldwork, following which all three engaged in independent analysis. The authors checked and discussed the coding of transcripts of each other, so ensuring reliability of interpretation and enhancing analysis. Finally, the analysis agreed across the authorial team for each case was considered against the overarching research questions. This was then fed back to both commissioners of the research and an advisory board of professionals and

managers that constituted users of the research. This provided a check on the authenticity of our analysis (Yin, 1994).

Findings

The inter-organizational challenge for work and employment relations

Driven by marketization and performance management, fragmentation of healthcare delivery ensued with consequent challenges for work and employment relations. At the start of the projects there were expectations of the lead clinicians that funding for the new genetics nurse role would be sustained following the end of the pilot. However, some acute hospitals within which genetics was to be mainstreamed were reluctant to take on responsibility for employing the genetics nurse. Wary of changes in government policy, and moving towards greater responsibility for their revenue associated with marketization, executive management at Carcity, for example, were reported to have refused to issue employment contracts for what they regarded as 'temporary' positions. The eventual outcome of protracted negotiations around the employment contract issue was that genetics nurses delivered service in the mainstream area within the acute hospital, but were physically located in (i.e. desk space) and had their contracts held by the regional genetics centre. This had adverse consequences for enactment of their role:

It was unclear who was managing me. The nurse manager within the mainstream area in which I was located didn't seem to know or care about what I was doing. I wasn't part of her team because I wasn't employed by the hospital and she let me just get on with it. What eventually emerged was that the genetic counsellor in the regional genetics centre took on the role of monitoring the quality of service we provided. However, even then, a major problem remained in that I didn't have a physical base since I wasn't employed by the hospital. I was seen as a visitor. As a consequence it made it very difficult to build up relationships to deliver the integrated service.

(Genetics nurse, Carcity)

There are a number of issues reflected in the quote above. First, there is the issue of monitoring clinical quality, which presents a considerable work and employment relations challenge, to which we return later. Second, building

relationships with clinicians appears an important early role for the genetics nurse, but one which, as we discuss in the next empirical section about inter-professional relationships, is subject to hierarchy. Finally, and most importantly, the relationship between regional genetics centres and acute hospitals in which mainstream services are located is crucial in implementing boundary spanning roles. We now discuss this further.

Linked to the organizational fragmentation that resulted from government policy, rivalry existed between regional genetics centres and acute hospitals regarding the allocation of resources to support delivery of genetics services. This was most evident in Bicity, where the genetics nurse became embroiled in a political battle between genetics and mainstream clinicians, with the regional genetics centre keen to keep resources to control the delivery of genetics services:

The biggest hurdle I had was trying to get people involved across the two hospitals in the city. The specialist, regional genetics service is located at this site and traditionally enjoyed the specialist resource for genetics service. The mainstream service is mainly at the other site, although we do a bit of the mainstream service here. The proposal for the pilot genetics services, which crossed the city, was driven by the regional genetics centre from this hospital, but for services that would be located at the other hospital. Both sides were worried about losing resource at first, which made my role very difficult. You have to pussy-foot around these clinicians since one word out of place and you upset the apple cart between the two organizations.

(Genetics nurse, Bicity)

Buttressing competition between regional genetics centres and acute hospitals within which mainstream services were located, were current financial constraints imposed by government. Influenced by the nature of genetics services, which are longer-term and preventive, regional genetics centres particularly felt under threat:

If genetics becomes more mainstreamed and we're just an add-on to a mainstream disease bid, then we might well end up getting nothing, partly because we just get lost in against the headline person making the bid. Then, if the full amount of funding isn't allocated, we'll probably be one of the things that is chopped off. We're not an acute speciality, we have no immediate impact and we can wait, whereas you know somebody having a heart attack can't wait.

(Business manager, Regional genetics centre, Shipcity)

Retaining genetics services within the jurisdiction and control of regional genetics centres was presented as a matter of ensuring clinical quality in all cases. The role of regional genetics centres in supervising clinical quality was accepted by genetics nurses:

I guess sometimes you come across cases where it's kind of borderline between one level and the other. I didn't want a cut off point, I didn't want to say well actually that cancer was over 40 and it might have been 42 so no, no you're not at risk. Our Friday meetings have been helpful here, where we have our clinical supervision meeting with the genetic counsellor. We discuss cases we just cannot work out or that we think will be referrals upwards and we just sit and talk through them.

(Genetics nurse, Shipcity)

Within Shipcity, not only was the genetics nurse managed by the regional genetics centre, but the genetics service she provided was located within the regional genetics centre, rather than in the mainstream disease area. On the one hand, the genetics nurse was meant to act as a conduit for the genetics clinician and counsellor to engage mainstream clinicians in the pilot project, through referring patients and supporting efforts to sustain funding. On the other, location of service in the regional genetics centre limited the boundary spanning role of genetics nurses and their influence over mainstream clinicians with whom they interacted. That the influence of genetics nurses was limited only became apparent as the project was implemented, since in the bid for funding, as with other projects, much was made of the network arrangements that facilitated boundary-crossing. The genetics clinician admitted a political imperative to sustain service, which led him and the network facilitator to represent the service as a 'network' to funders:

When it comes to the local development planning process through which the commissioners sign up to funding the service, we say to the commissioners, 'you know the surgeons have all signed up to this', although we don't ask them [the surgeons] whether they want to sign up to it!

(Genetics clinician, Shipcity)

Implicated in the above are inter-professional relations, particularly the clinician dominance over nurses. Buttressing the effect of the pilot project design, this further constrained the boundary spanning role of the genetics

nurse. In our next empirical section, we discuss these further, focusing upon the interaction of clinicians with genetics nurses. We also consider intra-professional relations, focusing upon the interaction of mainstream nurse-managers with genetics nurses.

Inter- and intra-professional challenge for work and employment relations

Inter- and intra-professional challenges of implementing the new genetics nurse role were evident during the recruitment phase. Applicants for the new post were thin on the ground because it did not fit with institutionalized grading arrangements in nursing:

This post required a degree of autonomy that nurses may find frightening. They are used to a greater degree of supervision from medical staff. Yet here they are asked to make decisions off their own back to a greater extent and behave more assertively in their interactions with doctors. The job needed to be at a higher grade and even then you are drawing from a few, very experienced and confident senior nurses. It was going to suit someone who had done a more research focused nurse role, than a ward based nurse, because in research there is a higher degree of independent decision-making.

(Genetics nurse, Carcity)

The suggestion in the above statement is that nurses are socialized into roles where they are dependent upon others' clinical decisions, rather than exercise clinical judgement themselves. And even where genetics nurses were recruited who were prepared to exercise autonomy, they found their opportunities for this significantly constrained by other stakeholders.

The enactment of a more autonomous role for genetics nurses was particularly constrained by the expectations of mainstream clinicians for a more traditional working relationship, where genetics nurses' roles were governed by the clinicians. First, the role of the nurses in mainstreaming genetics appeared of scant interest to mainstream clinicians, who remained focused upon their own services:

Nurses have to fight to gain recognition from hospital clinicians. We need to highlight to the clinicians that our role and genetics is worthy of their attention. I describe my job as a one woman battle to continue the momentum of the project and prevent hospital clinicians going off at a tangent and doing their own thing.

(Genetics nurse, Bicity)

Second, the everyday working relationship between mainstream clinicians and genetics nurses was one of professional hierarchy:

They don't see me as a helping hand. Instead, they stress that they are the doctor, they're doing the treatment, they've made a diagnosis and this is their recommendations. I'm just the nurse who communicates this to the patient.

(Genetics nurse, Bicity)

This meant genetics nurses can only work to a certain level in their interactions with mainstream clinicians within the network. For example, genetics nurses were limited in the extent to which they could educate mainstream medical doctors about genetics. As evident in the quotes above, traditional professional hierarchy dictated that this was best left to the expert clinician:

Me going to talk to mainstream clinicians is fine because they will listen to those people who come to inform them of clinical developments if they perceive them as experts.

(Genetics clinician, Shipcity)

Within Carcity, the relationship between one of the genetics nurses and the mainstream clinician became rather fractious. The genetics nurse had worked abroad previously in a less hierarchical context that allowed her to set up and run a nurse-led genetics clinic. On her return to England, she took up her present post and expected a high degree of autonomy in her role as genetics nurse to develop a care pathway that crossed genetics and the mainstream disease area and deliver joint clinics with the mainstream clinician:

Nursing has traditionally been subservient to medicine. I don't work like that. I have leadership tendencies and can work very clearly hand-in-hand with clinicians. And if they get uppity, then I can get uppity back.

(Genetics nurse, Carcity)

Her expectations were shattered early in her new position. Before the project-end she moved on to an academic post:

The clinicians demand their input is privileged in the development of the care pathway. All I do is merely co-ordinate their inputs and keep referring back to them as the pathway is developed. Meanwhile, the clinicians regard themselves as the experts in genetics and they think they can deliver the service and take it forward without my assistance.

(Genetics nurse, Carcity)

The limiting influence of the mainstream clinicians upon the new role extended to the department from which the genetics nurse moved. In some cases, there was a lack of support for the move:

The [genetics nurse] post was advertised as full-time, but I had to negotiate a part-time secondment from my mainstream area because the clinicians in the area didn't want me to go full-time.

(Genetics nurse, Bicity)

With respect to intra-professional relationships, two interactions were significant: the genetics nurse and mainstream nurse manager; the genetics nurse and genetics counsellor.

In our previous empirical section, we noted the nurse manager within the mainstream disease area in Carcity took little notice or responsibility for the genetics nurse. In Unicity, the nursing manager was even less supportive. In the face of constrained resources for mainstream services, first, the nurse manager attempted to deny the genetics nurse physical space (i.e. desk space) within the mainstream area and suggested she be better located in the regional genetics centre. Second, she attempted to leverage intra-professional hierarchy to load the genetics nurse with activity contributing towards mainstream service, at the expense of genetics service. The role of the nurse manager as a conduit for information and as the appraiser for the genetics nurse allowed this to happen:

On the one hand, she put me down saying, 'what does your role entail?' On the other, she then has to appraise me, which I find crazy, because she doesn't know what I'm doing on a day to day basis. Yet, I am reliant on the nurse manager and need to go to the departmental nurse meetings since, if I didn't, I wouldn't find out what was happening in the area.

(Genetics nurse, Unicity)

On being informed that the delivery of genetics services was being squeezed by the demands of the mainstream nurse manager, the genetics clinician intervened to ensure the intent of the original bid was realized. This relied, to some extent, on the good relationship that the genetics and mainstream clinician enjoyed, with the clinicians able 'to pull rank' on the nurse manager and ensure the requirements of mainstream service delivery did not drive out any genetics component of service.

However, as evident in our previous empirical section, the role of the genetics nurse remained 'squeezed' by other actors. Besides genetics and

mainstream clinicians, we highlighted how nurses were managed by genetic counsellors. Reflecting the influence of the genetic counsellor in limiting the autonomy of genetics nurses, it was interesting to see how the role of the genetics nurses expanded in the absence of the genetic counsellor:

When I first started the job I was supposed to be the genetics nurse and there was supposed to be a genetic counsellor but because they couldn't appoint a genetic counsellor, the role sort of expanded so I'm taking some of that. So talking to the genetics clinician and getting some more experience in genetics, I'm able to sit with patients and start doing family histories.

(Genetics nurse, Unicity)

Enhancement of the genetics nurse role to encompass genetics counselling required the support of the genetics clinician. Whilst genetics clinicians remained keen to control the genetics nurse to ensure services remained under the remit of the regional genetics centre and, of course, ensure clinical quality, they appeared comfortable with ceding delivery of genetic counselling services to genetics nurses:

This [failure to appoint genetic counsellor] has opened our eyes to the fact that someone with a mainstream disease background, when trained in certain aspects of genetic counselling and inherited disease, will actually do genetic counselling very well. Following our failure to recruit a genetic counsellor, the genetics nurse is developing her counselling role very well. Generally, a mainstream nurse specialist that developed a special interest in genetic disease could probably do the job exceptionally well without having to be formally trained as a genetic counsellor. As long as there was clearly somebody overseeing what they were doing.

(Genetics clinician, Unicity)

In summary, inter-professional and intra-professional relations linked to inter-organizational relations set limits upon the role of the genetics nurse. However, the way inter-professional and intra-professional relations impacted upon the genetics nurse was nuanced rather than monolithic, dependent upon the inter-dependence of roles and threats perceived by other actors to genetics nurses enacting a more autonomous and non-traditional role. In our discussion, we examine our empirics to draw out theoretical generalizations regarding work and employment relations challenges.

Discussion

There are two key inter-related dimensions of the work and employment relations challenges arising from our study: the effect of policy and the professional institution upon new forms of organizing service delivery. The effect of institutions plays out similarly across cases. Whether genetics nurses are controlled by mainstream clinicians or senior nurses in acute hospitals (with genetics clinicians or genetic counsellors in regional centres exercising control 'from a distance'), or whether nurses are under the direct control of genetics clinicians or genetic counsellors in regional centres, the enactment of an autonomous role proves difficult for nurses. Evident in all our cases is the interaction of the inter-organizational challenge (between regional genetics centres and acute hospitals), the intra-professional challenge (between genetics and mainstream clinicians; between genetics and mainstream nurses; between genetic counsellors and genetics nurses) and the hierarchical inter-professional conflict (between clinicians and genetics nurses).

In some cases, notably Shipcity, despite aspirations from the funding body (Department of Health) to 'shift the balance of power' and enact a more autonomous role for genetics nurses, the project was designed to converge with traditional arrangements for 'outreach' working in the first place with the regional genetics centre maintaining control. Yet simultaneously, there appeared a desire from the genetics clinician to embed genetics services in mainstream services. This particular mainstream clinical area was well institutionalized as an area for resource and a high political priority. We suggest it made political sense for the genetics clinician to 'piggyback' genetics services within wider provision in this area to sustain future funding. As discussed further within our conclusion, this calls into question whether a new organizational form is implemented at all. In part, claims by the genetics clinician that a 'network' would deliver the genetics service in the Shipcity case were made to secure and sustain funding, whilst traditional inter-organizational and inter-/intra-professional arrangements remained. The micro-politics of securing funding through claims that services will be networked reinforce uncertainty and difficulty faced by the genetics nurse charged with enacting a boundary-spanning role.

Securing funding was rendered more important by government policy, which was increasingly financially parsimonious. This impacted upon inter-organizational relationships, with a consequent negative impact upon the development of novel roles for nurses. For example, in Bicity, the regional genetics service was located in one hospital, but the mainstream service into which genetics was to be integrated was located in another hospital. Regional genetics was keen to control genetics services, and with it the management

of the genetics nurse, thus constraining her autonomy. At the same time, clinicians in the mainstream area within the neighbouring acute hospital were disinclined to allow the regional genetics any influence over mainstream services traditionally provided by them, and did not accept any expert input from the genetics nurse to mainstream service provision.

Marketization drives competition between organizations for resources. Traditionally, regional genetics centres have been the main beneficiaries from funding allocated to genetics. The intent of the 'mainstreaming genetics' initiative is one where, potentially, limited funding for genetics services is redirected, for example, to acute hospitals. As exemplified in the commentary by the business manager within Shipcity's regional genetics centre, this may threaten both genetics clinicians and genetic counsellors who are located at the regional centre, though overall resources devoted to healthcare may be used more effectively, for example, there may be fewer inappropriate referrals to expensive, specialist genetics services through broader, mainstream knowledge of referral criteria of those at risk from inherited conditions.⁴ Nevertheless, we should not underestimate the siloed nature of healthcare provision, exacerbated by traditional funding arrangements, and endemic 'tribalism' linked to professional specialization (Bate, 2000). Consequently, in response to any threat of genetics resources redistribution, particularly in Carcity, regional genetics centres attempt to re-assert control through monitoring the performance and managing the genetics nurses. The genetics clinician or genetic counsellor within the regional genetics centre educates (normative control) and manages the genetics nurses (more coercive control – e.g. over quality) delivering services in the acute hospital.

To emphasize a crucial point within our analysis, the model that genetics clinicians and genetic counsellors pursue is one of an outreach, rather than a more integrated mainstream service. This has consequences for work and employment relationships. Those in regional genetics centres seek to dominate the network relationship through their control of the genetics nurse, rather than give up power to clinicians or nurse managers in the acute hospital. This has consequences at the individual level for genetics nurses. The autonomy of genetics nurses to develop service and work across boundaries between the regional genetics centre and the acute hospital is significantly constrained within our cases.

Further 'squeezing' the autonomous role of nurses in Carcity, mainstream clinicians in the acute hospital see an opportunity to gain resources for the hospital, which are traditionally allocated to regional genetics services, through attempting to control the genetics nurse. Meanwhile, in Unicity, affected by financial constraints, the mainstream nurse manager within the acute hospital lays claim to scarce nursing resource and attempts

to extend the mainstream workload of the genetics nurse at the expense of genetics services. Again, we see the work and employment relations challenges associated with inter-organizational relations are buttressed by the impact of professional hierarchy.

Further illustrating the interaction of inter-organizational and inter-professional boundaries, within Carcity, mainstream clinicians in the acute hospital rein in the role of genetics nurses by requiring them to merely co-ordinate and administer clinician input to developing care pathways, rather than develop these care pathways and then consult clinicians: i.e. nurse discretion is restricted. Within 'joint' clinics, mainstream clinicians take the lead role and this includes diagnosis and management of the genetics component of care. This restricts the jurisdiction of genetics nurses over care so that a more traditional role of subservience to clinicians is enacted. In the face of her frustration about the limits of her role, the genetics nurse in Carcity left to take up an academic post before the project end.

There is a paradox in our cases. There is a desire, highlighted in the Carcity and Bicity cases, by both mainstream clinicians in acute hospitals and those genetics clinicians and counsellors located in regional centres, to control genetics nurses. At the same time, employment relations problems, such as the absence of formal performance management and reluctance to take on employment contracts, arise as a consequence of policy-driven, organizational fragmentation. Having long experience of temporary interest in new policy directions, notably illustrated in Carcity, organizations make business and service development decisions based on the knowledge that interest and funding for newer services will fade in the long term. Professionals and managers in hospitals appear reluctant to formally take up performance management responsibility and issue employment contracts. To do so would commit them to labour cost without any certainty funding would follow.

Intra-professional relations within nursing further contribute to the uncertainty genetics nurses experience in their role. Genetics is marginal, if recognized at all, within the nursing profession. The integration of nurses with their more 'mainstream' peers proves difficult to realize. Consequently, the nurse managers in mainstream areas are reported to leave genetics nurses to their own devices, as in Carcity, or in the case of Unicity, pull genetics nurses back into the mainstream, because they don't understand and/or recognize the value of genetics as a specialist nursing area.

This leaves nurses somewhat 'at the mercy' of experts located in the regional centre for guidance and support. The need for strict supervision of a more autonomous nurse role by the genetics clinicians was emphasized to ensure quality of service. Attempts by genetics specialists in the regional centre to control genetics nurses and rein in a more autonomous role may

well be required to limit risk and deliver 'safe' services for patients. We note this is accepted by genetics nurses, who, in the course of their socialization as nurses, may develop a reluctance to enact autonomy to the extent envisaged by designers of the boundary-crossing roles associated with new organizational forms.

Regarding the enhancement of the genetics nurses' role, the influence of genetic counsellors may also be potentially significant on the basis the genetics nurse might move into their area of jurisdiction. However, broader labour market conditions, illustrated in the case of Unicity, where it is revealed that genetics counsellors are in short supply, mean genetics counsellors may be willing to cede some of their lower level responsibilities to genetics nurses.

In short, the 'dark side' of new organizational forms of delivering service is evident within our study, with significant work and employment relations challenges associated with the development of new roles. Those genetics nurses in organizational and professional boundary-spanning positions experience uncertainty, ambiguity and frustration in their attempts to enact their novel roles.

Having emphasized this, in Unicity and Shipcity, we glimpse an antecedent to a more effective network. In Unicity, the relationship between the mainstream clinician and the genetics clinician in the regional centre appears one of trust, reciprocity and mutual understanding. Meanwhile, in Shipcity, the genetics clinician from the outset judged there to be synergy rather than threat from collaboration with the mainstream disease area, given pre-existing, institutionalized provision of funding. He sought to embed the provision of genetics services within the pre-existing mainstream network arrangements for service delivery. In Shipcity, the inter-organizational rivalry apparent in other cases was relatively absent.

Conclusion

Theoretically, through integrating organization studies/HRM and public administration literatures, our study contributes towards understanding work and employment relations challenges of new forms of organization, which consider the interaction of institutions, organizations and individuals (Marchington et al., 2005). More specifically, our study theoretically contributes to understanding the implementation of networks in healthcare (Ferlie & McGivern, 2003; Ferlie & Pettigrew, 1996; Goodwin et al., 2004; Hyde et al., 2005, 2006a, 2006b). Focusing upon professionals subject to the demands of changing organizational forms, we highlight the 'dark side'

of networks, in line with a burgeoning organization studies/HRM literature (see Marchington et al., 2005) and a public administration literature (see O'Toole & Meier, 2004).

Our study emphasizes that inter- and intra-professional relations are tightly interwoven with inter-organizational relations within the healthcare setting, and these create challenges for work and employment relations. We show networks that involve professionals are likely to function differently to those involving unskilled workers, with the issue of professional control and organization central to analysis (Grugulis et al., 2003; Hyde et al., 2005, 2006a, 2006b). We highlight the interdependence of professional groups, with power differentials characterizing relationships, in any move towards expanding or enhancing the role of any one profession (Nancarrow & Borthwick, 2005).

The effect of policy and professional institutions upon work and employment relations is emphasized within our study. Further research might usefully adopt an explicitly neo-institutional approach (DiMaggio & Powell, 1991; Scott, 2001) to theorize the link between regulatory, normative and cultural-cognitive institutions upon work and employment relations in the healthcare setting.

The overall effect of policy and professional institutions upon the 'network' in our study is one that leads us to question whether healthcare services in genetics are delivered via a 'network' at all. The synergies between organizational units that are characteristic of classical definitions of the network concept (see Powell, 1990) appear limited. Part of the explanation is the variety of arrangements covered by the term 'network' (Grugulis et al., 2003). More sceptical analysts have argued that the contemporary transformation of organizational forms is overstated (Grimshaw et al., 2005a). Organizations have always been networks with hierarchy a property of a network's structure (Barley & Kunda, 2001). Our definition of network, emphasizing how networks may include hierarchical arrays (O'Toole & Meier, 2004), is consistent with this. In short, we accept the concept of network is a pluralist one, including its use as a seductive soundbite or vehicle to enhance legitimacy in pursuit of resources, with our case representing a 'hybrid' rather than 'pure' network form (Grimshaw et al., 2005a). We also suggest, to secure legitimacy, 'network' forms of organization are promoted by those who seek funding, but within any new arrangements, traditional patterns of inter-organizational and inter-/intra-professional relations remain. There is further scope for exploring and classifying the variety of network forms that exist.

On this basis, we claim our analysis applies to healthcare delivery, whether services are conceived as a 'network' or not. Healthcare in general relies upon effective cross-boundary working and professional collaboration,

for example, one patient may move through a primary care organization, acute hospital and regional centre, and interact with different professionals for the treatment of a single condition. In enacting any boundary-crossing role, professionals, particularly those in less powerful occupations, may face similar challenges to our case of genetics nurses working within a 'network'. Further research might investigate this, including work and employment relations challenges associated with networks that cross primary care organizations, as well as acute hospitals and regional centres.

Within our study, we note antecedents to the enactment of new organizational forms and development of associated boundary-spanning roles. This comes in the form of trust, reciprocity and mutual understanding between professionals, located in different organizations, within the network. Whilst this article did not explore this in detail, the interaction of trust and power at a local level may be crucial in implementing networks (Bachmann, 2001, 2003). There is scope for further research to examine the trust-power relationship associated with the development of new organizational forms that breach institutionalized boundaries. We also note the effect of policy upon collaboration across organizational boundaries with the impact of funding regimes upon new forms of organization highlighted (Currie & Suhomlinova, 2006).

We argue our key findings are transferable beyond healthcare delivery, to other public sector contexts. Work and employment relations challenges emerge within our study as a consequence of inconsistent government policy and differential professional power. This is likely to extend to other public sector settings beyond England, subject to government policy and the influence of powerful professions.

Our key findings may also apply to private sector settings. In particular our findings apply where there are a number of powerful groups of professionals, with each cohering around a distinctive knowledge domain (Grugulis et al., 2003). This may be more so where private sector organizations are required to engage with those in the public sector, under private public partnership arrangements (Grimshaw & Hebson, 2005). In this situation the effect of performance-orientated government policy upon work and employment relations may also be evident.

Regarding practical implications, we confirm policy appears rather inconsistent (Currie & Suhomlinova, 2006; Newman, 2001). As Rubery et al. (2002: 645–6) note: 'The development of simultaneously more fragmented and more networked organizational forms raises issues of how to understand potential conflicts and contradictions around the "employer" dimension to the employment relationship.' Policy-makers might reflect upon the translation of policy prescriptions at the local level to drive change in the organization and delivery of healthcare towards networks.

For policy-makers, our analysis raises questions of whether we should abandon ideas of network forms of organization of service delivery, merely acknowledge the 'dark side' of networks, or whether we can improve the way networks operate. A recent study by Tagliaventi and Mattarelli (2006) provides an illuminating comparison with our own case. Tagliaventi and Mattarelli are rather more optimistic regarding the development of networks across professional groups to deliver healthcare. They highlight the following antecedents for effective networks: working side-by-side; holding common organizational values; exhibiting organizational citizenship behaviour. In our case, the first of these holds to some extent with nurses working alongside mainstream clinicians on a day-to-day basis when delivering clinics, although we note their employment contract is held by a different organization. Given nurses and mainstream clinicians work for different organizations, common organizational values and citizenship behaviour may be difficult to develop. Tagliaventi and Mattarelli's study took place in a single hospital unit in Italy. In contrast our study took place in a policy context where organizations were fragmented by markets and a strong performance orientation. This was exacerbated by local organization of genetics pilots. Associated with this, there may be a temporal element to the development of effective relationships that two years (the period of funding for pilots) did not allow for. Perhaps some thought should be given to longer funding streams to support policy ambitions for networked service delivery so that relationships across professions and organizations develop. In summary networks might work where policy is less inconsistent and budget support for networks is sustained.

Consequently, we suggest a need for more fine-grained studies of the effect of disparate national level policy strands upon the implementation of networks in public services settings. Given the development of networks for delivering public services is an international phenomenon, research may extend beyond the specific case of England. The need for more comparative research is rendered more acute given a failure to appreciate the development of new organizational forms in different national contexts (Alonso & Lucio, 2006; Rubery & Grimshaw, 2003).

Notes

- 1 We follow the definition of network by O'Toole and Meier (2004: 682): 'a pattern of interdependence among social actors in which at least a portion of the links are framed in terms of something other than superior-subordinate relations. Parts of a network may include hierarchical arrays, but at least some of the portions of the pattern are linked in another fashion. Networks may include multiple organizations or parts of organizations.'

- 2 For reasons of confidentiality, confirmed by the formal NHS ethics approval process, we do not divulge details of the specialist disease area.
- 3 HR practitioners were not identified as significantly engaged with the network. The role of HR practitioners in implementing new organizational forms is interesting in itself but lies outside the scope of this article.
- 4 Genetics clinicians may welcome this to some extent, given the risk, following increased awareness of genetics conditions, that they could be swamped with inappropriate referrals unless measures are taken.

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Graeme Currie, PhD, is Professor of Public Services Management at Nottingham University Business School. Currently, he leads two research programmes focused upon mainstreaming genetics healthcare and evaluating the delivery of children's services through organizational networks. His main research interests lie with leadership, knowledge management, workforce reconfiguration and translation of innovation in healthcare. His work has been widely published, more recently in *Public Administration*, *Journal of Management Studies* and *Human Relations*. In a previous life, Graeme worked in organization development within the NHS and personnel for the former car manufacturer, Rover Cars.

[E-mail: graeme.currie@nottingham.ac.uk]

Rachael Finn, PhD, is Research Fellow at the Institute for Science and Society, University of Nottingham. Her current interests focus upon the issue of inter-professional working, including teamwork, culture, knowledge, power and safety in changing institutional environments, with particular reference to the healthcare context. Her work utilizes qualitative methodologies, including ethnographic and discourse analytic approaches. Her work has been published in *Human Relations*, *Public Money and Management*, *Journal of Health Organization and Management*, and *Work, Employment and Society*.

[E-mail: rachael.finn@nottingham.ac.uk]

Graham Martin, MSc, is a research fellow at the Institute for Science and Society, Nottingham, and a part-time doctoral student. With a background in geography, his research has covered wide-ranging issues in the management and delivery of health and social care. His work has appeared in journals such as *Social Policy & Administration*, *Social Science & Medicine* and *Sociology of Health & Illness*.